BASELINE → TREATMENT → FOLLOW-UP

Testing across both treatment phases

Your team books tests and reviews their results. Imaging and tumour-marker timing is individualised; no universal interval is mandated here.

Check	Before starting	Combination cycles 1–4	Transition to nivolumab alone	Maintenance	After treatment
Blood count, kidneys, liver, glucose and thyroid	Baseline results	Before every cycle	Review results and symptoms before changing phase	Before every maintenance cycle	Follow persistent abnormalities
Symptoms and other hormones	Discuss autoimmune disease, infections, medicines and earlier immune treatment	Assess immune symptoms; cortisol if indicated	A phase change does not remove immune risks	Report symptoms immediately; extra tests as indicated	Monitoring for at least five months after the last dose
Treatment and observation	Confirm MSI-H/dMMR result and correct treatment line	Nivolumab first; then ipilimumab; vital signs and post-infusion observation	First maintenance dose three weeks after final combination	Confirm selected dose, route and two-/four-week interval	Retain the immunotherapy alert card
Scans and response	Agree assessment plan	Scans and markers as directed	Review benefit and suitability for maintenance	Review benefit, tolerability and duration	Agree next care and follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00900 v1a official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Nivolumab + ipilimumab · first-line colorectal: treatment and checks

First-line unresectable or metastatic MSI-H/dMMR colorectal cancer

Cycles 1–4: nivolumab 240 mg, then ipilimumab 1 mg/kg, on Day 1 every 21 days. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review. Nivolumab alone: 240 mg intravenous (IV) every two weeks or 480 mg IV every four weeks.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Combination cycles 1–4	Transition to nivolumab alone	Maintenance	After treatment
Blood count, kidneys, liver, glucose and thyroid NCCP 00900	Baseline results	Before every cycle	Review results and symptoms before changing phase	Before every maintenance cycle	Follow persistent abnormalities
Symptoms and other hormones NCCP 00900	Discuss autoimmune disease, infections, medicines and earlier immune treatment	Assess immune symptoms; cortisol if indicated	A phase change does not remove immune risks	Report symptoms immediately; extra tests as indicated	Monitoring for at least five months after the last dose
Treatment and observation NCCP 00900	Confirm MSI-H/dMMR result and correct treatment line	Nivolumab first; then ipilimumab; vital signs and post-infusion observation	First maintenance dose three weeks after final combination	Confirm selected dose, route and two-/four-week interval	Retain the immunotherapy alert card
Scans and response NCCP 00900	Agree assessment plan	Scans and markers as directed	Review benefit and suitability for maintenance	Review benefit, tolerability and duration	Agree next care and follow-up

What each check is for

1 Cycles 1–4: two immunotherapies

Every 21 days

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR deoxyribonucleic acid (the cell's genetic material) (DNA)-repair feature. This is a first-line treatment for unresectable or metastatic cancer.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00900

2 Cycle 5 onwards: nivolumab alone

Team-selected maintenance schedule

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR DNA-repair feature. This is a first-line treatment for unresectable or metastatic cancer.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00900

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.35